

Project Charter

Project Name: Clinical Management System (CMS)

Project Manager: Kay Brooks (kbrooks@abcclinic.ca) & Rachel Bryant (rbryant@abcclinic.ca)

Customer: ABC Clinic Operations & Medical Staff

Project Sponsor: John Doe, Clinic Manager (jdoe@abcclinic.ca)

Project Start/End (projected): November 1, 2026 – July 31, 2027 (9 Months)

Project Overview:

This project will develop and deploy a comprehensive, paperless **Clinical Management System (CMS)** for the new ABC family medicine and walk-in clinic in the Greater Toronto Area. The purpose of this system is to streamline clinic workflows serving 1,500–2,000 patients weekly, automate patient scheduling and walk-in queue management with triage prioritization, secure electronic medical records in compliance with government privacy regulations, enable electronic billing (Ministry of Health and Accounts Receivable for non-covered/insurance services), and facilitate electronic exchanges for prescriptions, lab requisitions, and specialist referrals.

Objectives:

- **Regulatory Compliance:** Securely store and safeguard patient electronic health records in strict compliance with governmental privacy and confidentiality mandates.
- **Workflow & Operations Automation:** Automate appointment booking, walk-in queueing, triage jumping, and status tracking (booked, arrived, checked in, checked out, LWT, no-show).
- **Real-time Verification:** Provide real-time Health Card verification directly linked with the Ministry of Health system upon patient check-in.
- **Electronic Diagnostics & Referrals:** Enable doctors to generate and transmit electronic/printed prescriptions, lab requisitions (and receive lab results), and specialist referrals.
- **Financial & Payroll Support:** Support direct government billing for physicians, manage Accounts Receivable/Payable for non-covered cash/insurance items, and track non-salaried staff working hours for payroll processing.
- **Timely Delivery & Budget Adherence:** Complete full system deployment within the \$1,000,000 budget and the 9-month timeframe.

Key Assumptions:

- The budget limit is fixed at \$1,000,000 and the project timeline cannot exceed 9 months.
 - External entities (Ministry of Health, third-party medical labs, pharmacies, specialist offices, and accounting firm) will provide compatible electronic interfaces or APIs.
 - Staffing levels will remain approximately 10 physicians, 6 nurses, 8 receptionists, and 2 assistant managers.
 - System interfaces will be web-browser/cloud-based for accessibility across examination rooms and front desk stations.
 - Paper records received (such as outside lab results or specialist notes) can be scanned and securely stored in the system database.
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Stakeholders and Responsibilities:

Stakeholder	Role	Responsibility	Signatures
John Doe	Clinic Manager / Project Sponsor	Project Vision, Budget Approval (\$1M), Scope Sign-off, Operations Alignment	<i>John Doe</i>
Kay Brooks	Co-Project Manager	Project Planning, Monitoring, Execution, Resource Management, Deliverables	<i>Kay Brooks</i>
Rachel Bryant	Co-Project Manager	Requirements Architecture, Testing Strategy, Risk Management, Deliverables	<i>Rachel Bryant</i>
Lead Physician	User Representative (Doctors)	Clinical Workflow Functionality, EHR Specs, E-Prescriptions & Billing Needs	<i>Lead Physician</i>
Lead Nurse	User Representative (Nurses)	Vitals/Symptom Entry Functionality & Triage Workflow Requirements	<i>Lead Nurse</i>
Head Receptionist	User Representative (Front Desk)	Scheduling, Walk-in Queue, Health Card Verification & Payment Forms	<i>Head Receptionist</i>